Northern Devon Healthcare NHS Trust

Document Control

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Critical Care Admissions Guideline			
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0.3	Jan 2020	Review	Presented to the surgeons at the joint Anaesthesia/Surgery Governance meeting.
0.4	March 2020	Review	Presented it at the medical Grand Round in March and every consultant had a copy.
1.0	July 2020	Final	Approved at Divisional Governance and virtually noted at CAGG.
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1. Purpose

1.1. National Department of Health guidance (EL-96-20) informs decision making with regard to critical care admissions; this document was published in 1996 and provides general guidance only. This local guideline has been written, recognising that more recent evidence on critical care outcomes exists, that critical care is a limited resource, and that an understanding of the basis of ICU decision-making is important for referring specialities.

1.2. It should be acknowledged that the majority of admission decisions include an element of triage. The ethical principle of justice underpins these decisions. Notably, efforts are made to accommodate all appropriate patients, regardless of bed occupancy. This may involve activation of escalation plans. In very rare cases (e.g. COVID-19 pandemic) case numbers may exceed local, regional and, indeed, national critical care bed capacity. In such a scenario, triage and admissions criteria may be adjusted, with close involvement of the South West Critical Care Network and NHS England. *At all other times, admission decisions will be guided by the principles within this document, and will be applied equitably to all.*

1.3. Patients with limited functional capacity and physiological reserve cope poorly with the stress of critical illness, invasive organ support and, crucially, the (often prolonged) recovery period afterwards. Additional evidence suggests that patients with limited function, who survive their critical illness, are often left more disabled subsequently.

1.4. There is scientific rationale in favour of a 4 MET minimum peak effort tolerance as one criterion for admission, as patients with lower peak function are less likely to have a good outcome, particularly in the longer term. Examples of 4 MET activities include: walking up a flight of stairs without stopping, moderate housework (e.g. vacuuming or mopping) and walking on the flat at 4 mph without stopping.

1.5. The guideline applies to all Specialities where a referral to critical care is being considered.

1.6. Implementation of this guideline will ensure that:

- Referring specialties understand the criteria for admission
- Referring specialities are able to make more timely and appropriate escalation decisions

In all cases, referral and escalation decisions should be made at a senior level.

2. Decision to Admit

2.1. Patients must not be refused appropriate treatment solely on the basis of lack of Critical Care Unit bed capacity.

2.2. As per local and national guidance, the final decision regarding all ICU admissions, including those for single-organ only support, rests with the ICU consultant. This will, however, involve close liaison with the patient, relatives, admitting team and, on occasion, other ICU consultant colleagues.

2.3. All discussions and decisions, including the rationale for the decision, must be clearly documented in the medical notes.

Early consideration of resuscitation status is recommended, particularly in those patients with a ceiling of ward-based care agreed.

3. Admission Criteria

Both of the following criteria will be applied to the majority of referrals for admission to the ICU:

The below criteria do assume that the patient with capacity has consented to escalation of care. In those without capacity, decisions should be made by the medical team in the best interests of the patient (unless a valid and applicable advance directive or power of attorney exists).

3.1. A - Those with reversible pathology and a treatment plan agreed in outline

Those patients with a terminal diagnosis and/or malignancy will be considered, with close involvement of the patient and parent speciality team, bearing in mind that the recovery period from critical illness is frequently prolonged and may alter the balance of benefits and burdens of critical care. Particular care should be taken in situations involving a terminal diagnosis and/or expected survival of less than 12 months; examples include: decompensated alcoholic liver disease with multi-organ failure, metastatic cancer, cardiac failure, pulmonary fibrosis, progressive neuromuscular conditions, etc. In some cases, where patients have end-stage disease, it may be appropriate to admit to ICU for stabilisation, if they have already been accepted for transfer to a tertiary centre, e.g. accepted for tertiary organ support or transplant**.**

3.2. B - Those with reasonable functional capacity (ideally ≥ 4 METS, e.g. moderate house work, climbing stairs, walking on flat at 4 mph, mowing lawn, cycling)

It is accepted that functional capacity may be difficult to quantify in a proportion of patients (e.g. amputees, severe arthritis.) In this setting, and certainly where doubt exists, a period of critical care should be considered, with the trend in clinical condition and response to treatment guiding subsequent management decision-making. Early MDT discussion (e.g. 48-72 hours) is recommended in these cases.

Note that specific diagnoses and, likewise, age do not feature in the initial admission decision, as all patients meeting the criteria above should be considered. However, there is increasing evidence in the critical care literature that frailty is associated with poor outcomes in critical care. Consideration should therefore be given to routine frailty scoring and this should be taken into consideration as part of the decision making process.

4. Special Circumstances

4.1. Patients with significant chronic disease, who are offered long-term single organ support by a parent speciality (e.g. renal medicine) may be considered for single organ support on the ICU outside of the usual admission parameters described above.

Escalation to multi-organ support in this context will be subject to the same criteria as any other admission – and it may be determined that this would NOT be appropriate given the severity of the underlying disease and the impairment of functional status

4.2. Where the ICU is providing single organ support in this manner, every effort should be made by the parent team to prioritise early transfer to the destination unit (e.g. tertiary Renal Unit, Respiratory NIV bay, etc.) and regular review.

4.3. It should be appreciated that the majority of paediatric and other young patients with disabilities do *not* have co-existing cardiorespiratory disease, and as such should be assumed to have physiological reserve appropriate for admission to the ICU.

5. Monitoring compliance with, and the effectiveness of, the Guideline

5.1. Standards/ Key Performance Indicators

Key performance indicators comprise:

- Monitoring critical care performance using monthly data produced by ICNARC
- Learning from incidents and complaints
- Oversight and review of mortality and morbidity data

5.2. Process for Implementation and Monitoring Compliance and Effectiveness

Following consultation with speciality groups and approval of the policy it will be published on BOB and shared through governance pathways.

Policy needs to be shared through the speciality groups to ensure all rotational, permanent and temporary staff are aware of admission criteria.

Monitoring compliance with this policy will be the responsibility of the Critical Care Group. This will be undertaken by monthly monitoring, incidents and mortality and morbidity reviews.

Where non-compliance is identified, or referrals fall outside recommendations, support and advice will be provided to the speciality team.

6. Equality Impact Assessment

The author must include the Equality Impact Assessment Table and identify whether the policy has a positive or negative impact on any of the groups listed. The Author must make comment on how the policy makes this impact.

Group	Positive Impact	Negative Impact	No Impact	Comment
Age			X	
Disability	X			Specific reference made to disabled patients, whose disability may impair 'functional status' and who therefore should be assumed to have physiological reserve appropriate for ICU admission.

Gender			X	
Gender Reassignment			X	
Human Rights (rights to privacy, dignity, liberty and non-degrading treatment), marriage and civil partnership			X	
Pregnancy			X	
Maternity and Breastfeeding			X	
Race (ethnic origin)			X	
Religion (or belief)			X	
Sexual Orientation			X	

7. Associated Documentation

- Critical Care Operational Policy
- Contingency Plan for Increased Demand on Critical Care Services Policy
- Critical care internal professional standards